GCPP—142-C/8—5,00,000 Cps. (823)—20-6-2025.

[redacted]

Date: 14

[redacted]

Number: 9944 Age: 48 M F C C

Date 14 MAR 2026

℞

H/o fever x 2 weeks.

no H/o cough, cold, vomiting, abdominal pain, loose stools.

H/o generalised myalgia.

no H/o loss of weight.

No H/o hunger nocturnal

Advice

1. T. para 500 mg 1-0-1 x 3 days
2. Inj. para 2cc IM stat.
3. Review with reports.

Admission in FMW

To do

Typhoid
Dengue
Lepto
Scrub typhus.

[redacted]

Date and time :

COMPLAINTS :

C/o fever x 3 days.
H/o myalgia (+)
No H/o cough & cold.

No H/o vomiting / loose stool

Duration :

HISTORY :

No H/o Burning micturition

Pt was apparently normal 3 weeks Back after which she developed fever, which was insidious in onset, progressive in nature Low grade in nature. with no aggravating and relieving factors.

NO H/O vomiting, weight loss, evening rise in temperature, cough with expectoration.

H/O palpitation is there which was insidious in onset and low grade in nature.

N/H/O Loose stools, diarrhoea, etc.

Past History

NH/O TB, Asthma, Previous surgeries and N/H/O ~~pre~~ similar complaints in the past.

NHO DM, HTN, epilepsy, Asthma

AMINATION:

Family History

O/E

BP-110/70 mmHg
PR-78/min

N/H/O similar complaints in family members.

Personal History

G/E

Wt
Ht
BMI
Pt

consumes mixed diet.
Normal bowel and B bladder Habits.
NHO smoking, Alcohol, Tobacco consumption

menstrual History

married 29 years ago with LMP - 25/2/20 with irregular cycles.

Examination

General Examination → conscious, oriented

P - I - C - C - L - E -

S/E

P/A → soft NT
CNS - NFND

CVS - S1S2 normal
RS - BL AE ⊕

VISIONAL DIAGNOSIS: Enteric Fever

Adv -

PROGRESS AND TREATMENT SHEET

Date and Investigations	Progress	Instructions	Treatment and Diet
	To do CBC Urine RE widal Dengue serology ECG.		- TPR/BP charting - IVF 1 ⊙ NS @ 100ml/hr. - Inj. Tazim 1g IV TDS - Inj. pan 20 IV OD - T. pcm 650mg 1-1-1 - T. FLT 40mg 1-0-0 - Syp. sucralfate 10ml thrice

Date and Time (1)	I.V.F. (2)	Amount
4/3/26 @ 3.55pm		

COMMUNITY

INTAK

GCPP.—142-C/7—10,000 Cps.(1527)—12-9-2011.

[redacted]

INTAKE OUTPUT CHART

Date and Time (1)	I.V.F. (2)	Amount (3) ₹	Oral Fluids (4)	Amount (5) ₹	UOP (6)	Vomitus (7)	Aspiration (8)	Remarks (9)
14/3/26 @ 3.55pm	c/o headache & neck pain. BP: 112/80 mmHg. PR: 90bpm SpO2: 99% @ RA.						advice ① T. Diclo 50 mg stat.	
14/3/26 @ 7:43pm	c/o headache & fever. O/E - Febrile PR = 113 SpO2 = 98%.						Rx. - Inj PARA 1amp im stat	
15/3/26 6.25 Am	c/o headache & nausea PR - 103 SpO2 - 99% BP 98/mmHg						Rx. - Inj. DICLO 1amp im stat. - T. EMBIBET 4mg stat. - IVF 1 ⊙ NS @ 100ml/hr	

15/3/21
4.00pm

c/s/ [redacted]

c/o headache and fever.

Adv:

Inj - para 2cc 8-7 Start

Increase intake of oral fluid

[redacted]

S/E.
OVS
RLS
P/A

BP: 120/80 mmHg.
PR: 70/min

Todo
Repeat LBC Tomorrow

COMMUNIT

.e and Time (1)	I.V.
13/26	

GCPP.—142-C/7—10,000 Cps.(1527)—12-9-2011.

[redacted]

INTAKE OUTPUT CHART

Date and Time (1)	I.V.F. (2)	Amount (3)	Oral Fluids (4)	Amount (5)	UOP (6)	Vomitus (7)	Aspiration (8)	Remarks (9)
		₹		₹				
16/3/26 @ 1.00 am			c/o fever with chills & rigor.					
BP:	118/86							Adv:
PR:	112/min							
SpO2:	98%						T. Para 650 mg SOS.	

[redacted]

GCPP.—142-C/8—3,000 Cps. (1139)–4-7-2018.

[redacted]

DIET SHEET

Ward : Date :

Bed No.	I. P. No.	Patient's Name	Type of Diet	Remarks, if any
16/3/26 2 pm		Pt c/o Headache + fever		
				Advice
				① T. Para 650 ~~Stat~~
				② 100 NS IV Fluid
		c/s/B Dr. Vinothini (PG)		
17/3/26 6:40am		Pt. interviewed,		
			c/o Headache	
			Last fever episode yesterday evening.	
			Myalgia	
			Arthralgia	
				Advice
	BP-120/80 mmHg			℞ Inj. Tardom 1g iv 1-1-1
	PR-72 bpm			
				T. FMT 40mg 1-0-0 (B/F)
				T. Para 650mg SOS
				Syp. Sucralfate (B/F) 10ml 1-0-1

Adv
CBC
ophthal review

[redacted]

IPTO

Bed No	I P No	Patient's Name	Type of Diet	Remarks
		As NAD		18/3/26 @ 7:00 AM
		Vn 6/6 6/9		
		C PH 6/6 6/6		
		H/O using PG.		
		Refraction		
		17/3/26	IVF:	10 NS
		Pt before was		10 DNS
		headache		10 RL
		gen myalgia		75 ml/hr
		afebrile	by Tab pcm	1g iv 1-1
		Cy/Ex/ABD	T. pcm	650g 1-1
				PRN
			C. DMEZ	20 1-1
			Inj. Lucrafate	10 ml 1-1
			by Pcm in fever	
			Discharged	
17/3/26		Pt was not found for past 3 hours and was not present in the ward.		

Signature of Staff Nurse

18/3/26
@7:00AM

patient reviewed

O/E
PR: 108 bpm
BP: 128/90 mmHg
SpO_2: 99% @RA
GMR: 137 mg/dl

Advice

(D5) ① Inj. Paxim 1g IV 1-1
② Tab. para 650mg SOS
③ T. Omez 20mg 1-0-1

18/3/26

Con of AKI
c/o fever
O/E-Ge-Pallor
PR-104/min
BP-120/80 mmHg

Ab

[redacted]

CM/MD

GCPP—142 C/7—10,000 Cps (1521)

[redacted]

INTAKE OUTPUT CHART

Date and Time (1)	I.V.F. (2)	Amount (3)	Oral Fluids (4)	Amount (5)	UOP (6)	Vomitus (7)	Aspiration (8)	Remarks (9)
		₹		₹				
19/5/26 @ 5 a.m.		C/S/B casualty team. c/o fever since night. O/E: BP — 128/88 mm Hg; PR — 110/min; SpO2 — 97% @ RA; GMR — 141 mg/dl; S/E: CVS, CNS, RS, P/A — NAD.				Advice: ① 1 NS, 1 DNS, 1 RL @ 75 ml/hr; ② Inj. Taxim 1g IV 1-1-1; ③ T. PARA 650 mg SOS; ④ T. OMEZ 20 mg 1-0-1		

Dr. [illegible]

C/S/S Dr. PRADEEP, CMO

29/03/26

Fever spike yesterday -
pt. afebrile now

Fever was after chills
headache
generalized myalgia
No jt pain
No bleeding manifestations
No urinary discharge c/o
No vomiting / loose stools

Typhoid
Dengue
Scrub
Leptos
-ve

Cns: No sign of meningeal irritation

CVS
RS
} NAD

P/A Soft (R) Hypochondrium tender
epigastric tender +
Pts ulcer forms tender.
No McBurney pt tender

TC = 7600
P 27 L 68
Hb = 9

MCV ↓
MCHC ↓
MCH ↓

1) INV: IONS
IOBL
(Rs)

2) Inj Taxim 1g iv 1-0-1

3) T. para 500mg 1-0-1
(Rs)

4) Inj. PAN 40 iv od 1-0-0

5) Syp Sucralfate 10ml 1-0-1

6) T. Dicyclomine 1-0-1
(Rs)

Impression

C/c Febrile illness c̄ Abdominal pain & Lymphocytosis
- To r/o Abd. TB vs Enteric (Typhoid 3m/36 -ve)
Anemia (Iron def')
AFB

Plan: Refer to GH for
- USG-Abdomen

GCPP.—142-C/7—10,000

[redacted]

INTAKE OUTPUT CHART

20/3/26.

Date and Time (1)	I.V.F. (2)	Amount (3)	Oral Fluids (4)	Amount (5)	UOP (6)	Vomitus (7)	Aspiration (8)	Remarks (9)
		₹		₹				
19/3/26 @ 3pm			c/s/B:	casualty	team			
	O/E: PR: 109 bpm BP 110/80 mmHg SpO_2: 98% @RA GRBS: 216 mm/Hg					Adm	IVF 100 ml NS I.V stat	[redacted]
PO DO FBS PPBS								

CIS/B Casualty team

20/3/26.

@ 7:00am

c/o fever.

O/E:

PR - 90/min
BP - 126/86 mmHg
SpO_2 - 98% @ RA
GMR - 131

S/E:

CVS
CNS
RS
PA
} NAD.

Advice:

① 1⊙ NS.
1⊙ DNS
1⊙ RL
} @ 75ml/hr SOS

D7. ② Inj. Taxim 1g IV 1-1-1
③ T. Para 650mg SOS.
④ T. omez 20mg 1-0-1
5) T. BC 1-0-0

20/3/26
@4:00pm

CIS/B - casualty team
c/o Fever
c/o Neck pain

O/E -
BP - 130/90 mm Hg
PR - 82/min
SPO_2 - 96% @ RA

GCPP.—142-C/7—10,000 Cps

COMMUNITY HEALTH CENTRE, KARIKALAMPAKKAM

INTAKE OUTPUT CHART

21/03/26
7AM

Date and Time (1)	I.V.F. (2)	Amount (3)	Oral Fluids (4)	Amount (5)	UOP (6)	Vomitus (7)	Aspiration (8)	Remarks (9)
		₹		₹				

20/03/26
10pm

C/S/B Dr. Vanthini (MO)

c/o Neck pain a/w nape of neck swelling since morning.

H/o last fever episode yesterday night.

c/o myalgia

O/E - Neck
ROM - Free & full on extension & then ~~onto~~ (R) side.

Adv:
Inj. BC 2cc IM Stat
T. Para 500mg Stat
Tg paint dressing.

[redacted]

Tx:
X ray neck lateral view

21/03/26.
7AM

a/s/b Causalty Team
c/o Neck pain & swelling
Afebrile x 24 hrs

O/E:
BP - 110/80 mmHg
PR - 98 bpm
SpO_2 - 99% @RA

S/E:
CVS
CNS
RS
P/A
} NAD

Advice:

1) 1 ⊙ NS
1 ⊙ DNS
1 ⊙ RL
} @ 75ml/hr SOS

2) T. PARA 650mg SOS
3) T. OMEZ 20mg 1-0-1
4) T. BC 1-0-0
5) (to stop) Inj. TAXIM 1g IV HH
6) T. SERATIOPEPTIDASE 10mg 1-0-1
7) IG dressings
8) HOTPACK APPLICATION